AMERICAN ACADEMY OF
HOSPICE AND PALLIATIVE MEDICINE

February 23, 2026

The Honorable Robert F. Kennedy, Jr.
Secretary
Department of Health and Human Services
200 Independence Avenue, SW
Washington, DC 20201

RE: HHS Health Sector AI RFI [RIN 0955-AA13]

Dear Secretary Kennedy:

On behalf of the more than 5,000 members of the American Academy of Hospice and Palliative Medicine (AAHPM), thank you for the opportunity to respond to the Department of Health and Human Service (HHS) request for information (RFI) on accelerating the adoption and use of artificial intelligence (AI) as part of clinical care. AAHPM is the professional organization for physicians specializing in Hospice and Palliative Medicine (HPM). Our membership also includes nurses, social workers, spiritual care providers, and other health professionals deeply committed to improving quality of life for the expanding population of patients facing serious illness as well as their families and caregivers. Together, we strive to advance the field and ensure that patients across all communities and geographies have access to high-quality palliative and hospice care.

AAHPM appreciates that AI can have positive impact on the delivery of clinical care, including for patients with serious illness. However, we also recognize that there are inherent risks that come with use of AI when appropriate safeguards are not in place. We therefore appreciate the opportunity to provide the input below, which we believe could help to maximize the benefits of AI in serious illness care, while also ensuring that AI is used safely and responsibly.

AI in Serious Illness Care

AAHPM believes that the application of AI in serious illness care holds great promise in informing shared decision making, treatment planning, and the identification of necessary psychosocial and spiritual supports, all with the aim of ensuring that patients receive care that is consistent with their goals and preferences. It can also help to reduce provider burden, including through assistance with administrative tasks such as medical record documentation and coding and billing. Importantly, this promise extends to the delivery of hospice care for patients near the end of life, who (along with their families and caregivers) may be at one of the most stressful and vulnerable points in their lives.

In order for this promise to be realized, however, investments must be made to support development of AI for serious illness care. Too often, AI tools are targeted towards intervention-based care. In our members' experience, for example, ambient listening AI has been relatively effective in capturing discussions about procedures or imaging, but it does a poor job of extracting information from discussions about goals and values, which are central to the delivery of palliative care. AI that is designed to support serious illness care would ideally address these limitations.

To ensure that seriously ill patients and their palliative care teams are able to fully reap the benefits of AI, ***AAHPM urges HHS to support development of AI for use in serious illness care, including hospice care.*** Such AI should include non-device clinical AI that augments communications, goals-of-care documentation, symptom assessment, and care planning. AI tools that can be used in the delivery of virtual care – for example for assessing caregiver stress and functional decline virtually – would also be valuable. We also highlight the importance of caregiver involvement providing care to seriously ill patients, who often require assistance in day-to-day activities of daily living, not to mention accessing care or adhering to treatment plans. We therefore stress the importance of AI for serious illness care prioritizing caregiver-inclusive design, in order to reduce caregiver burden and optimize caregivers' experience.

Regulatory, Payment Policy, or Programmatic Design Changes to Prioritize

AAHPM appreciates HHS' interest in identifying regulatory, payment policy, and programmatic design changes to prioritize to incentivize the effective use of AI in clinical care, and we offer our recommendations below.

Coverage and Payment

To begin, we believe there is a need for clearer pathways for providing coverage and payment of AI non-device tools, including for the Medicare program. Under current Medicare rules, for example, AI applications used in clinical practice generally do not receive separate payment, but instead are reimbursed as part of existing service codes, leaving uncertainty as to whether physicians can recoup their costs for the use of AI tools. ***AAHPM therefore encourages HHS to establish clear policy that provides a pathway for AI tools to be covered and reimbursed appropriately.***

Patient and Caregiver Protections

We also emphasize the need to ensure there are adequate protections for patients and caregivers when services are delivered using AI – particularly when AI is used to inform treatment decisions or assist in insurance coverage determinations – and we recommend that HHS promulgate policies that ensure accountability in the delivery of services furnished using AI and support safe, effective, and high-quality

care. This includes policies requiring human oversight, upholding physician judgment, and establishing quality accountability when AI is used for anything other than administrative workflows.

For human oversight, at a minimum, HHS should require human-in-the-loop use of AI tools when used for clinical care – particularly when high-risk and high-impact treatment decisions are at stake. In such cases – for example decisions to withhold or withdraw life sustaining treatments – safeguards are critical, including requirements for appropriate clinical oversight by the physician or non-physician practitioner (NPP) directly involved in managing the patient's treatment and care plan. To ensure that physicians and other practitioners are incorporating human oversight, we recommend that requirements be put in place regarding documentation of such oversight when AI is used in the delivery of care. Similarly, we note that AI is increasingly being used by payers to make coverage determinations, for example through prior authorization processes. Human oversight is necessary in these cases as well, and HHS should restrict the use of AI to make automated denial decisions without human review.

We believe that human oversight is particularly important in serious illness care, especially at the end of life when AI algorithms may suggest poor outcomes or limited benefit of treatment options, and we underscore the importance of honoring independent clinical judgment as an important safeguard in protecting patients' access to care. Indeed, we believe that physicians maintain ultimate responsibility for treatment decisions, regardless of the use of AI, and therefore exercise of physician judgement should be paramount. HHS should therefore ensure that there are appropriate limits on the use of AI in making clinical decisions and clarify that AI should only be used to augment physician judgement, not replace it. And in no case should AI be able to impose restrictions on access to medically appropriate treatment or symptom management. We can imagine, for example, AI determining that a certain treatment may not be appropriate for a seriously ill patient given algorithms suggesting likely poor outcomes, or AI-based restrictions on the use of opioids or other medications appropriate for patients with serious illness that are outside of common use. We could also envision AI algorithms recommending against hospice election, for example, based on algorithms that conclude the patient has a prognosis greater than 6 months. In neither case should these algorithms be able to limit access to treatment choices that are in opposition to physician judgement and patient preferences. As an added precaution to protect physician judgment and support transparency and accountability, HHS should require audit trails and counterfactual explanations when AI is used for clinical decisions.

Finally, we recommend that HHS establish a framework for ensuring accountability for quality of care when AI is used in the delivery of care. For example, HHS should explore tying use of AI tools to quality measures. For serious illness care, this could include measures related to symptom improvement, documented care goals, and avoidance of burdensome transitions.

Care Team Protections

In addition to protections for patients and caregivers, ***we recommend a policy framework be established to offer protections for care teams.*** For example, our members have raised concerns that health systems may seek to use AI to unreasonably increase output and service delivery. AAHPM believes that AI should be used to augment care teams, and can be used to improve efficiency, but reasonable limits are necessary to protect the well-being of the care team and limit the risk of burnout.

Additionally, HHS should establish a policy framework for care teams' reasonable use of AI for low-risk activities, including those that support administrative workflows. In particular, we point to the use of ambient listening AI to support medical record documentation, which has been subject to litigation based on patient consent and privacy concerns. While we agree that privacy and security must be protected, we also recognize that current regulations may not keep pace with technological advancements. Updates to regulations to support reasonable use of AI in low-risk situations would protect care teams' ability to leverage AI to increase efficiency and reduce burden.

Evaluation of AI Tools

HHS requests input on evaluation of AI tools, which AAHPM agrees should be a priority. However, we highlight the need to evaluate AI in the context of the care being delivered. In particular, we note that ***AI for serious illness care should be evaluated on outcomes that are meaningful in such care delivery,*** including on metrics related to communication, prognostic framing, and care decisions. Metrics relating to imaging or diagnosis, for example, would not be as relevant for the delivery of serious illness care. Likewise, benchmarks for performance should also be specific to the care being delivered. Accordingly, we recommend that HHS support the development of benchmarks for serious illness care that consider the distribution of age, multimorbidity, and functional status reflected in seriously ill populations.

We also recommend that HHS support rigorous evaluation of AI tools both pre-deployment and post-deployment. Such evaluation should include human-centered user experience testing, bias audits on prognostic or triage recommendations for different populations, cybersecurity and data privacy checks for sensitive information, and routine drift monitoring and outcome tracking. We therefore recommend that HHS advance frameworks that would make clear when and how often these evaluations should be conducted and who (between provider, developer, or third-party auditor) is best positioned to conduct them. Effective analysis of AI tools would also require transparency on part of the developer in order to evaluate training inputs and applicability of AI outputs to all relevant patient populations.

Support for Private Sector Activities

HHS requests input on ways to best support private sector activities to promote innovative and effective AI use in clinical care, pointing to potential use of accreditation, certification, industry-driven testing, and credentialing.

AAHPM agrees that accreditation or certification requirements could support reliability of AI tools and would support HHS efforts to foster industry-driven programs. This support could be through grants or cooperative agreements, for example, and should include programs that focus on the use of AI for serious illness care. We recommend that accreditation or certification requirements fostered by HHS emphasize human oversight, documentation of limitations, and transparent escalation protocols.

Leveraging Interoperability to Accelerate AI Adoption

HHS seeks information on where enhanced interoperability would widen market opportunities, fuel research, and accelerate the development of AI for clinical care.

AAHPM agrees that greater interoperability would support expanded use of AI, but we highlight the need to ensure that interoperability requirements address serious illness care. As a starting point, we believe that standard application programming interfaces (APIs) should capture data elements relevant to serious illness care, including related to:

- Goals of care
- Advance directives, as well as information on physician orders for life-sustaining treatment (POLST)
- Primary decision-maker and/or healthcare proxy
- Symptom and burden scales
- Palliative performance status
- Caregiver needs
- Preferred place of care and death, as applicable

Research and Funding Priorities

HHS requests information on specific areas of AI research that HHS should prioritize to accelerate the adoption of AI as part of clinical care.

AAHPM believes research addressing the use of AI for serious illness care must be prioritized. This includes research into AI tools that would improve the quality of provider-patient communication and decision support, as well as AI tools to support symptom monitoring and medication safety. We also recommend research to assess AI implementation, for example the impacts of AI implementation on provider burden and clinical workflows, as well as the impacts to patients and caregivers. Research in these areas will help to address major evidence gaps that are likely to have consequential impacts in the delivery of care for patients with serious illness.

Conclusion

Thank you, again, for considering our comments in response to the RFI on AI in clinical care. As noted above, we believe advancements in this area can support more efficient and effective serious illness care, as long as appropriate safeguards are in place, and we look forward to the advancements that further investments and policy guidance can foster. We would be happy to continue to serve as a resource for HHS as you undertake this important work. Please feel free to reach out to Wendy Chill, Senior Director, Health Policy and Government Relations, with any questions at wchill@aahpm.org or (847) 375-6733.

Sincerely,

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